

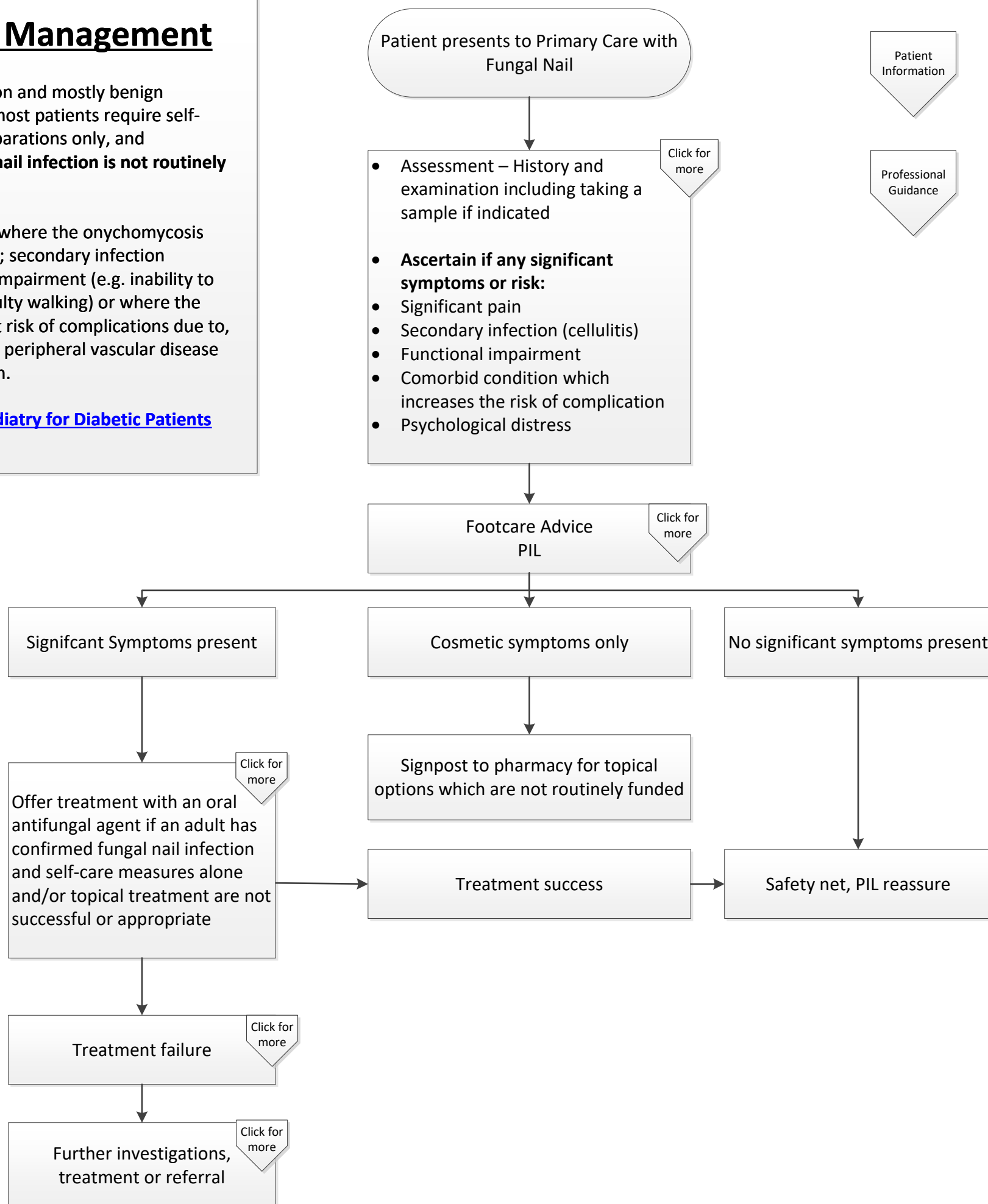
Fungal Nail Management

Fungal nail is a common and mostly benign condition. Therefore most patients require self-care or pharmacy preparations only, and **treatment for fungal nail infection is not routinely funded by the NHS.**

Exceptions to this are where the onychomycosis causes significant pain; secondary infection (cellulitis); functional impairment (e.g. inability to use footwear or difficulty walking) or where the patient is at significant risk of complications due to, for example, diabetes, peripheral vascular disease or immunosuppression.

See [Foot Health & Podiatry for Diabetic Patients](#) pathway

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Assessment and History

Assessment of suspected fungal nail infection should include:

- Asking about symptoms such as pain and discomfort; difficulty wearing footwear and walking; psychosocial impact; any co-morbidities or risk factors for infection; any family or household members affected.
- Classifying the type of onychomycosis, if possible.
- Examining for fungal skin infection at other sites.
- Arranging nail clippings and/or scrapings for fungal microscopy and culture to confirm the diagnosis and underlying cause, if the person wishes to start antifungal treatment.

Consider differentials:

- Psoriasis
- Lichen planus
- Bacterial infection
- Onychogryphosis
- Onycholysis
- Viral warts
- Subungual melanoma

Please see NICE CKS(Diagnosis/Assessment) in Professional Guidance (click more option)

Footcare Advice

- Keep nails trimmed short and filed down. Avoid sharing toenail clippers with family members.
- Wear well-fitting non-occlusive shoes, without high heels or narrow toes. Consider replacing old footwear which could be contaminated with fungal spores.
- Wear cotton, absorbent socks.
- Maintain good foot hygiene, including prompt treatment of any associated tinea pedis. See the CKS topic on Fungal skin infection - foot for more information.
- Wear protective footwear when using communal bathing places, locker rooms, and gymnasiums, to avoid re-exposure.
- Avoid prolonged or frequent exposure to warm, damp conditions if possible.
- Avoid trauma to the nails if possible.
- Provide information on sources of advice and support - **Please see Patient information (click for more) for PILS**

Oral Treatment

- See nice guidance for appropriate treatments
- Do not start treatment without a positive scrapping result
- Interpret the results with caution, as false-negative rates for culture may be as high as 30%.
- Advise the person a negative test result cannot definitively exclude fungal nail infection.

Inconclusive scrapings:

- Arrange for repeat samples to be taken if the result is negative and there is a high clinical suspicion of fungal nail infection.

Consider referral (to skin health triage/dermatology) if:

- Child under 18 needing oral medication
- Uncertain diagnosis
- unsuccessful treatment
- Immunocompromised patient
- There is co-existent nail disease, such as psoriasis or lichen planus.

Referral to a podiatrist may be considered if:

- Thickened toenails cause discomfort when walking.
- There is nail trauma due to the person's footwear.
- Deformed toenails are traumatizing adjacent toes.

Please see NICE CKS for treatment options

Treatment Failure

If there are signs of treatment failure, the following measures can be considered:

- Managing any underlying cause.
- Re-sampling for nail clippings and/or scrapings for fungal microscopy and culture.
- Combination treatment with a topical and oral antifungal agent.
- Referral to skin health triage

Patient Information

- British Association of Dermatologists (*information leaflet*)- <https://www.bad.org.uk/pils/fungal-infections-of-the-nails/>
- NHS – Fungal Nail Infection (*Do's/Don'ts – Information leaflet*) <https://www.nhs.uk/conditions/fungal-nail-infection/>
- Fungal Nail Clipping - <https://www.youtube.com/watch?v=9gbLf9lo2Do>

Professional Guidance – Website Information

- NICE CKS (diagnosis/assessment) **<https://cks.nice.org.uk/topics/fungal-nail-infection/diagnosis/assessment/>**
- NICE CKS (management/ treatment options) (**<https://cks.nice.org.uk/topics/fungal-nail-infection/management/management/>**
- Dermnet NZ - **<https://dermnetnz.org/topics/fungal-nail-infections>**
- British Association of Dermatologists (Professional) - **<https://onlinelibrary.wiley.com/doi/10.1111/bjd.13358>**

Consider arranging referral to a podiatrist if:

Consider arranging referral to a podiatrist if:

- Thickened toenails cause discomfort when walking.
- There is nail trauma due to the person's footwear.
- Deformed toenails are traumatizing adjacent toes.
- Consider arranging referral to a paediatric dermatologist if:
- Oral antifungal treatment is being considered for a child.

Consider arranging referral to a dermatologist if:

- The diagnosis is uncertain.
- Treatment in primary care is unsuccessful.
- There is co-existent nail disease, such as psoriasis or lichen planus.
- The person is immunocompromised, depending on clinical judgement.